

Title: ADHERE: Time to First Improvement in Stage A	
Project Owner: Marilla Cherveny	Date: September 11, 2025
Agency Contact(s): N/A	

Purpose:

This data on file packet is the reference for the following information:

In quantitative and qualitative research, as well as multiple ad boards, healthcare providers (HCPs) repeatedly perceive time to initial improvement as a key differentiator for VYVGART Hytrulo. It also has the highest recall rate, compared with other data points.

Medical and commercial materials contain a figure showing the time to first improvement on aINCAT, I-RODS, or grip strength in Stage A, with the data called out for the 25th, 50th, and 75th percentiles. While the 25th and 50th percentile data points have now been published (EAN 2024 and Allen manuscript, respectively), data for the 75th percentile remains unpublished.

V2.0 (September 2025) update:

- Renewing DOF packet for continued use of supportive data for the 75th percentile of time to first improvement
- Removed 25th and 50th percentile as these are now supported by published sources
 - 25th percentile: van Doorn P, Allen J, Basta I, et al. Efficacy and safety of efgartigimod PH20 in chronic inflammatory demyelinating polyneuropathy: results of ADHERE/ADHERE+. Oral Presentation at: European Academy of Neurology (EAN); 29 June-02 July 2024; Helsinki, Finland.
 - 50th percentile: Allen JA, Lin J, Basta I, et al. Safety, tolerability, and efficacy of subcutaneous efgartigimod in patients with chronic inflammatory demyelinating polyradiculoneuropathy (ADHERE): a multicentre, randomised-withdrawal, double-blind, placebo-controlled, phase 2 trial. *Lancet Neurol.* 2024; 23(10):1013–24. [https://doi.org/10.1016/s1474-4422\(24\)00309-0](https://doi.org/10.1016/s1474-4422(24)00309-0).

Use(s):

Please describe the manner in which the following data will be utilized (e.g. multiple assets, multiple agencies, etc.):

These data may be used in the following example medical materials, including but not limited to:

- MSL field materials
- Medical Information documents
- Field readiness guide
- Advisory boards
- Symposia
- HEOR materials

These data may be used in the following example commercial materials, including but not limited to:

- Pre-approval information exchange (PIE) and payer value proposition (PVP) decks
- Commercial interactive visual aid (CIVA)
- Frequently asked questions (FAQs)
- HCP marketing materials
- Patient marketing materials
- Speaker decks

Content:

As shown in Table 14.2.3.1 below:

- A total of 322 patients received efgartigimod rHuPH20 SC in Stage A. There were 250 patients with first improvement during Stage A.
- First improvement is defined as an aINCAT improvement (decrease) of ≥ 1 point, an I-RODS improvement (increase) of ≥ 4 points, or a mean grip strength improvement (increase) of ≥ 8 kPa, regardless of the scale patients deteriorated on during the run-in period.
 - Note that this differs from the criteria for identifying responders who entered the randomized withdrawal period (Stage B).
- In ADHERE Stage A, 75% of patients demonstrated first clinical improvement in aINCAT (improvement of ≥ 1 point), I-RODS (improvement of ≥ 4 points), or mean grip strength (improvement of ≥ 8 kPa) in 57.0 days (95% CI, 44.0-63.0).

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TABLE 14.2.3.1: TIME TO FIRST IMPROVEMENT - KAPLAN-MEIER

ANALYSIS SET: STAGE A SAFETY

	EFG PH20 SC (N=322)
25th PERCENTILE (95% CI) (DAYS)	9.0 (8.0; 9.0)
50th PERCENTILE (95% CI) (DAYS)	22.0 (15.0; 23.0)
75th PERCENTILE (95% CI) (DAYS)	57.0 (44.0; 63.0)
ESTIMATED PERCENTAGE OF FIRST IMPROVEMENT AT WEEK 12 (95% CI)	94.1 (89.2; 97.2)
NUMBER ASSESSED	322
NUMBER OF EVENTS [1]	250 (77.6 %)
NUMBER CENSORED	72 (22.4 %)
COMPLETED STAGE A TREATMENT	7 (2.2 %)
DISCONTINUED FROM TREATMENT IN STAGE A	65 (20.2 %)
ALL EVENTS REQUIRED FOR PRIMARY ANALYSIS ACHIEVED	12 (3.7 %)
DISCONTINUED FOR ANY OTHER REASONS	53 (16.5 %)

NC = NOT CALCULATED.

[1] PARTICIPANTS WITH FIRST IMPROVEMENT DURING STAGE A.

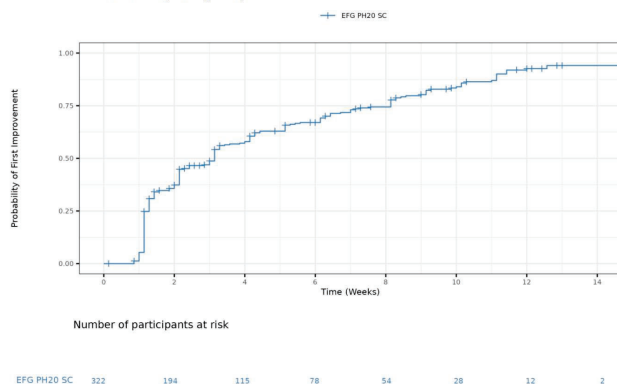
FIRST IMPROVEMENT IS DEFINED AS HAVING IMPROVEMENT ON THE SAME EFFICACY PARAMETER AT 2 CONSECUTIVE VISITS, REGARDLESS OF THE NUMBER OF DOSES OF IMP THE PARTICIPANT RECEIVED. IMPROVEMENT CAN OCCUR ON ANY OF THE FOLLOWING EFFICACY PARAMETERS, REGARDLESS OF THE PARTICIPANT'S WAY OF ECMD DETERIORATION: ADJUSTED INCAT SCORE (DECREASE ≥ 1 POINT VERSUS STAGE A BASELINE), I-RODS (INCREASE ≥ 4 POINTS VERSUS STAGE A BASELINE), OR GRIP STRENGTH (INCREASE ≥ 8 KPA VERSUS STAGE A BASELINE IN AT LEAST ONE HAND). THE DENOMINATOR FOR THE PERCENTAGE CALCULATIONS IS THE TOTAL NUMBER OF PARTICIPANTS IN THE STAGE A SAFETY ANALYSIS SET.

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first clinical improvement in any of these parameters was 9.0 (8.0-9.0) days in 25% of participants (Figure 5).

Figure 5: Kaplan-Meier Plot of the Time to First Improvement (SAF-A) (Stage A – Secondary Endpoint)



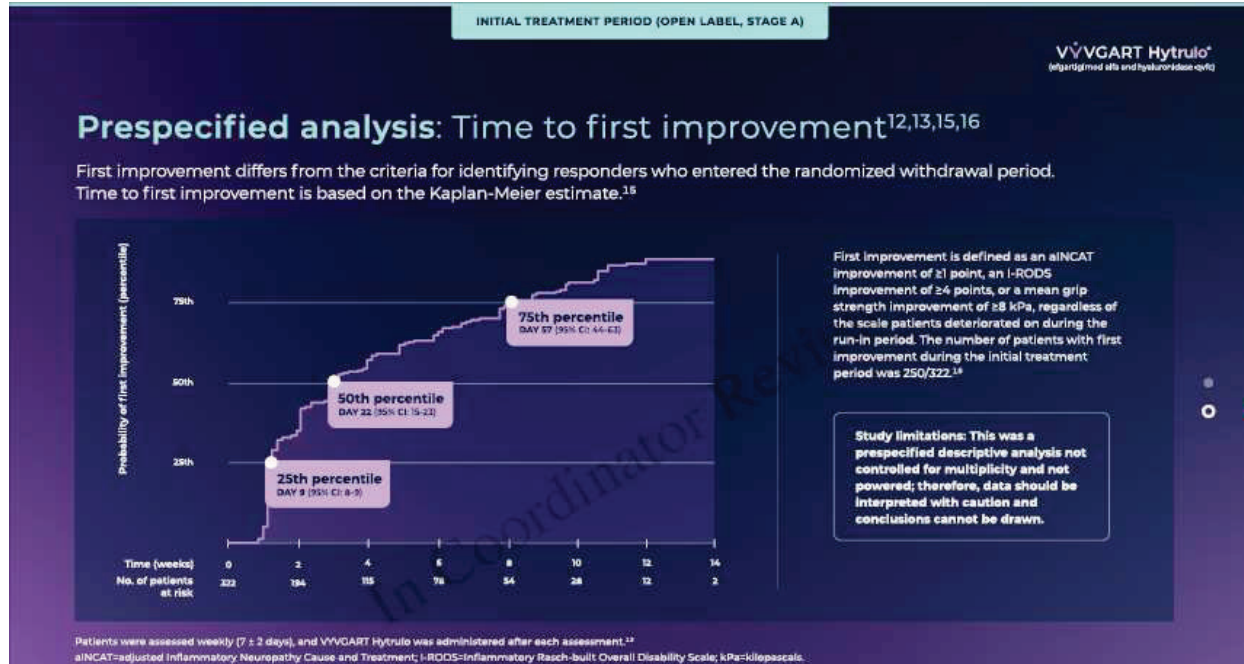
Source: Table 14.2.3.1

EFG PH20 SC=efgartigimod for SC administration coformulated with rHuPH20; SC=subcutaneous[ly];

rHuPH20=recombinant human hyaluronidase PH20.

Note: Participants who completed Stage A without confirmed ECI or discontinued treatment in Stage A were censored at the date of the last available aINCAT, I-RODS, or mean grip strength assessment.

Example data presentation below:



Note to content creators: please refer to the following published sources to support the 25th and 50th percentile data points:

- 25th percentile: van Doorn P, Allen J, Basta I, et al. Efficacy and safety of efgartigimod PH20 in chronic inflammatory demyelinating polyneuropathy: results of ADHERE/ADHERE+. Oral Presentation at: European Academy of Neurology (EAN); 29 June-02 July 2024; Helsinki, Finland.
- 50th percentile: Allen JA, Lin J, Basta I, et al. Safety, tolerability, and efficacy of subcutaneous efgartigimod in patients with chronic inflammatory demyelinating polyradiculoneuropathy (ADHERE): a multicentre, randomised-withdrawal, double-blind, placebo-controlled, phase 2 trial. *Lancet Neurol.* 2024; 23(10):1013–24. [https://doi.org/10.1016/s1474-4422\(24\)00309-0](https://doi.org/10.1016/s1474-4422(24)00309-0).

Reference:

ARGX-113-1802 Final Analysis

- Table 14.2.3.1 Time to First Improvement – Kaplan-Meier
- Figure 5 Kaplan-Meier Plot of the Time to First Improvement (SAF-A) (Stage A – Secondary Endpoint)

Talking Points:

- A total of 322 patients received efgartigimod rHuPH20 SC in Stage A. There were 250 patients with first improvement during Stage A.
- First improvement is defined as an aINCAT improvement (decrease) of ≥ 1 point, an I-RODS improvement (increase) of ≥ 4 points, or a mean grip strength improvement (increase) of ≥ 8 kPa, regardless of the scale patients deteriorated on during the run-in period.
 - Note that this differs from the criteria for identifying responders who entered the randomized withdrawal period (Stage B).
- In ADHERE Stage A, 75% of patients demonstrated first clinical improvement in aINCAT (improvement of ≥ 1 point), I-RODS (improvement of ≥ 4 points), or mean grip strength (improvement of ≥ 8 kPa) in 57.0 days (95% CI, 44.0-63.0).

Checklist for additional requirements to appropriately describe analyses:

X Type of analysis Prespecified analysis

X Analysis set Stage A safety

☐ Mean at baseline

☐ Confounders, if applicable

X Limitations of analysis This was a prespecified descriptive analysis not controlled for multiplicity and not powered; therefore, data should be interpreted with caution and conclusions cannot be drawn.

Approval	
U.S. Medical Leadership	Name: Josh Bryson <i>Josh Bryson</i>
	Date: 2025-09-12 04:45 CEST
EMA Medical Leadership	Name: Roland Derwand <i>Roland Derwand</i>
	Date: 2025-09-14 17:17 CEST
Chief Medical Officer	Name: Luc Truyen <i>Luc Truyen</i>
	Date: 2025-09-15 12:56 CEST